Data Sharing Agreement between London Borough of Islington and London Borough of Camden re the delivery of Public Health Functions

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1)Introduction to the Sharing

This Data Sharing Agreement [DSA] documents how the parties to this agreement will share personal data about Staff and service users to enable both Parties' single service Public Health functions and the jointly delivered Public Health functions (described in Appendix 1) to be delivered. The Parties note there is an umbrella Memorandum of Understanding between them for Public Health functions, with this DSA covering the Data Protection requirements of the service. By signing this DSA the Parties agree to accept the conditions set out in this document, according to their statutory and professional responsibilities, and agree to adhere to the procedures described.

This Agreement has been developed to:

- Define the specific purposes for which the Parties have agreed to share information.
- Outline the Personal and Special Category to be shared.
- Set out the lawful basis conditions under UK GDPR and Data Protection Act 2018 through which the information is shared, including reference to the Human Rights Act 1998
- Stipulate the roles and procedures that will support the processing/sharing of information between Parties.
- Describe how the rights of the data subject(s) will be protected as stipulated under the data protection legislation.
- Confirm the lead council for Joint Data Controller matters
- Describe the security procedures necessary to ensure that compliance with responsibilities under data protection legislation and Party-specific security requirements.
- Describe how this arrangement will be monitored and reviewed.
- To illustrate the flow of information from referral through processing and outcome.

Parties to this agreement cannot amend or add appendices unless agreed as part of a formal review. It is expected that each party will have procedures, processes and policies sitting underneath this agreement, for their respective organisations. These will, for example, describe the specific processes for secure transfer of data.

1.1. Responsibilities of Parties

The Parties are registered with the ICO as Data Controllers. Each Party is a Data Controller for the data processed by each Party individually under this DSA. In respect of the jointly delivered work through shared posts described in Appendix 1 the Parties are Joint Data Controllers. It is agreed that for the jointly delivered work only, London Borough of Islington has primary responsibility for complying with UK GDPR obligations in respect of transparency obligations and individuals' rights. For the other Public Health work, not jointly delivered, each Party will comply with its obligations around transparency and individuals' rights.

Both Parties confirm that they comply with data protection legislation by:

- having a lawful basis for processing and sharing personal data.
- ensuring data quality.
- storing and sharing information securely, with access management controls.
- having policies and procedures for compliance with data protection legislation including for managing data subject rights and complaints, identifying and managing data breaches/incidents and retention & disposal.
- ensuring that mandatory training is undertaken regularly by their employees to ensure they are clear and up to date on their responsibilities. Every individual must uphold the principles of this agreement and overarching confidentiality, and seek advice from the relevant Data Protection Officer when necessary.

- undertaking appropriate data protection due diligence checks with any contractors/data processors they employ, and ensuring that a written agreement is in place with each data processor, and that all data processors will be bound by this agreement.
- having written processes for the processing of data to ensure employees use and share personal data in line with data protection law, the data protection principles, and this agreement.

Organisations and their staff must consult the organisation's Data Protection Officer/Information Governance Manager and/or Caldicott Guardian if they are unsure at any point in the processing and sharing of personal data.

Each Partner must ensure that there are appropriate written contracts or agreements with employees, agency staff, volunteers etc. These must include requirements to ensure compliance with policies which include confidentiality. Each Partner must ensure that suitable vetting has taken place through standard employee checks and DBS checks as needed.

In respect of data disclosed to either Party under this agreement the Parties agree that each Party is a Data Controller and therefore is responsible for onward sharing and other handling of such data in accordance with Data Protection laws.

1.2. Review and Termination of this DSA

A review of this information sharing agreement will take place yearly, unless otherwise agreed by the Parties' Data Protection Officers. The aim of the review will be to ensure the purposes are still relevant, the scope has not slipped, the benefits to the data subjects and organisations are being realised, and the procedures followed for information security are effective. Changes in legislation and developments in the areas of public sector data sharing will be considered as and when they arise.

In the event of termination of this agreement each party may continue to hold information originating from other parties for which they are Data Controller. Termination requires three month's written Notice (email is acceptable) from the terminating Partner's Director of Public Health to the other Partner's Director of Public Health, with such Notice being copied to the both Partners' DPOs at the contact emails in Section 5 below.

2. Purpose and Benefits

2.1. Purpose and Benefits

This agreement covers the sharing of information by and between the listed parties for the purpose of delivering an effective Public Health service across both Boroughs. In respect of the jointly delivered Public Health functions (described in Appendix 1) the sharing will allow the service to be delivered effectively and efficiently, attaining the economies of scale intended by the shared function. In respect of data sharing for the functions delivered independently by each Party, the sharing will allow each Party to better deliver its Public Health functions to its residents resulting in better value for money, and shared expertise leading to higher quality outcomes for residents.

The sharing will allow both partners to better deliver their statutory series and will allow more effective use of resources, giving more joined up working which increases efficiency. The public will benefit from this approach, as services will be delivered in a more streamlined way that utilises the information shared to provide better tailored and appropriate services.

The benefits of this DSA are to:

- Cover the sharing of information for
- Sets parameters for sharing personal data and clearly identifies the responsibilities of organisations.
- Identify the correct lawful basis to share personal information.
- Ensure information is shared whenever there is a requirement to do so.
- Enables authorities to share data on performance, quality assurance, learning and impact analysis.

2.2. Lawful Basis

The sharing of information must comply with the law relating to confidentiality, data protection and human rights. Having a legitimate purpose for sharing information is an important part of meeting those legal requirements. This is a complex area and each Partner must take their own decisions and seek advice from their organisation's Data Protection Officer/Information Governance Manager and/or Caldicott Guardian.

Articles 6, 9 and 10 of the UK GDPR set out the acceptable conditions for the processing and sharing of personal, special category, and criminal data. The conditions relevant in the UK GDPR to data processed under this agreement are below.
Local authorities engaged in the provision of Health and Social Care are required to do so under one or more of the following acts, and their amended or updated regulations:

- Care Act 2014
- Children Act 2004
- Health and Social Care Act 2012
- National Health Service Act 2016

The Health and Social Care Act 2012 gives Local Authorities powers to perform public health functions, including commissioning and collaborating on a range of public health services to improve and protect the health of the local population and reduce health inequalities, and for advising the commissioners of local NHS services as well as the health and wellbeing boards on these matters.

Local authorities engaged in Public Health provision are required to do so under one or more of the following regulations and their successors:

- Public Health (Control of Disease) Act 1984
- Statistics and Registration Service Act (2007) Section 42(4)
- Health and Social Care Act (2012) section 287
- Health Service (Control of Patient Information) Regulations 2002 regulation 3

These acts create and delegate ministerial functions in the public interest, and the Data Protection Act 2018 section 8 specifically permits these as being purposes for Article 6 1(e) of UK GDPR – tasks in the public interest.

Where the parties are to share personal information that relates to a person's private or family life, the right to privacy in Article 8 of the European Convention on Human Rights (ECHR) will be applicable.

Personal data:

For the sharing of data between the Parties for general Public Health matters and for the shared service: Article 6(1) (c) legal duty and 6(1)(e) processing is necessary for the performance of a task carried out in the public interest or for the parties' official functions, and the task has a clear basis in law.

For London Borough of Islington processing data on the behalf of London Borough of Camden for the shared service: Article 6(1) (c) legal duty and 6(1)(e) processing is necessary for the performance of a task carried out in the public interest or for the parties' official functions, and the task has a clear basis in law, and where the processing is providing a service direct to the public for which is consent is required, consent under article 6(1)(a).

Special Category Data:

For the sharing of data between the Parties for general Public Health matters and for the shared service:

- Art 9(g) Reasons of substantial public interest (with a basis in law), and permitted under the Data Protection Act 2018 Section 10 (Substantial Public Interest) as it meets the following conditions set out in Schedule 1 Part 2 para 6 (Statutory and government purposes) and para 18 (Safeguarding of children and of individuals at risk)
- Art 9(h) Health or social care (with a basis in law), and permitted under the Data Protection Act 2018 Section 10 (Health or social care purposes) as it meets the following conditions set out in

Schedule 1 Part 1 (2)(2) (d) the provision of health care or treatment, and (e) Health or social care purposes

- Art 9 (i) Public health (with a basis in law), and permitted under the Data Protection Act 2018 Section 10 the Data Protection Act 2018 (Public Health) as it meets the following conditions set out in Schedule 1 Part 1 (3) (a)(b) Public Health
- For London Borough of Islington processing data on the behalf of London Borough of Camden for the shared service: the same as above, plus where the processing is providing a service direct to the public for which is consent is required, explicit consent under article 9(a)

2.3. Consent

Consent is not the lawful basis the public sector organisations use for processing information shared under this agreement, except where the Shared posts are providing health services directly to the public. It is noted that where consent is not the lawful basis for processing, consent does not need to be sought to share, and thus the concept of "overriding consent" is a misconception. Where consent is required the UK GDPR requirements for this will be met.

2.4. Proportionality and necessity

Proportionality, data minimisation, necessity and not being excessive are factors to be taken into consideration when deciding whether to share personal information. In making the decision, employees must weigh up what might happen as a result of the information being shared against what might happen if it is not, and apply their professional judgement. It is for this reason professionals must ensure they comply with Article 5(1)(c) and share the adequate and relevant information, and limit that information to what is necessary for the achieving of the DSA aims.

2.5. Freedom of Information

The Freedom of Information Act 2000 gives all individuals the right to access official information held by a public authority (the Environmental Information Regulations 2004 also allow access to data. For ease of drafting, FOI is used to cover both legislation). Limited exemptions may apply and all public authorities must ensure they have recognised procedures in place for administering requests of this nature.

All requests for FOI will be directed through the relevant organisations' FOI processes. Each party will seek advice/opinion from the other parties where there is concern about that information being released and any impact it is likely to have. The final decision to disclose or not will lie with the party who received the request as the legal duty lies with them. In order to ensure that the authority in receipt of the FOIA request is able to respond within the statutory deadline, any request for assistance or information made to partner authorities should be made and processed within two working days, and any data exchange completed within seven working days.

It is encouraged that all parties proactively publish this document. It may also be disclosed to the public under FOI.

3. Individuals

Organisations processing personal data are required to begin with the ethos of Data Protection by Design and Default (also known as Privacy by Design (PbD)). This means that we must consider and uphold the privacy of an individual's data before we begin and throughout the processing taking place.

Each party agrees that they have undertaken a DPIA (Data Protection Impact Assessment), where they feel the processing meets the legislative criteria for a DPIA.

3.1. Right to be informed – Privacy notices

Where personal data is created or received by one of the parties, they are responsible, as required by law, for making the data subject(s) aware within a reasonable time frame that the organisation holds the data, what they will do with it, how long they will keep it, and who they will share it with (such as under this DSA). This is normally done through a privacy notice, whether written or verbal. Organisations agree

that they will adhere to the transparency requirements of the UK GDPR and will issue appropriate notices which inform the data subject that the information will be shared with the parties under this agreement. For the Shared Service Posts, London Borough of Islington will act as the lead Data Controller for providing Privacy and Transparency information.

3.2. Data subject rights requests and complaints

Each organisation must have in place appropriate policies and processes to handle data subject requests made in line with data protection law, to ensure they are responded to within deadline and in an appropriate manner. Requests include; right of access, right to rectification, right to erasure, right to restrict processing, right to data portability, right to object and rights related to automated decision making including profiling.

If an individual successfully requests the erasure or limitation of use of their data (right to erasure, right to rectification, right to restrict processing, right to object), the party that has been informed by the data subject will communicate this to the other parties where relevant and appropriate. In each case each party is responsible for securely disposing of such information or limiting its processing.

Each party must have clear, fair and objective complaint procedures. Any complaints from individuals how their data is being processed or shared will be handled under the policy and processes of the organisation concerned.

For the Shared Service Posts, London Borough of Islington will act as the lead Data Controller for Data Subject Rights requests and complaints.

3.3. Data subjects

The data subjects will be:

- Staff employed by either Partner Public Health service
- Members of the public engaging with the Shared Public Health functions
- Professionals involved with either of the above

4. Data

4.1. The data to be shared

Data that will be shared includes (but is not limited to):

- name and contact details
- address
- age/date of birth
- Telephone number
- NHS Number
- Gender
- Race, Nationality and Religious or philosophical beliefs (to allow programme to be delivered in a culturally appropriate way)
- Special Educational Needs and Disabilities
- Health needs assessments
- Personal data relating to smoking cessation
- Births and deaths data
- Hospital episode statistics

- Sexual health data
- Opinion data of residents collected via focus groups, surveys and other forms of engagement
- Physical or mental health or condition (to allow risk assessments to be made and necessary actions taken)
- Linked health and social care datasets

4.2. Storing and handling information securely

Information must be stored and shared lawfully and securely. Special category data may need a higher level of security. The employee/organisation sharing the information must choose the most appropriate secure method of transfer and be responsible for its safe delivery.

Electronic records:

Organisations may have different electronic methods for storing and sharing information securely. Some have local restrictions which block access to information shared using specific tools.

Parties must make sure the chosen method is suitably secure and that access is only provided to those who need it, and only to the data needed.

Unencrypted email (i.e. sent in plain text over the public internet) must not be used to share information under this DSA.

Sharing methods that may be appropriate include:

- **Email encryption tools** where the email and attachments are encrypted from named sender to named recipient (e.g. Microsoft 365 Message Encryption; Egress Protect)
- **Encryption via Transport Layer Security** (TLS) where the email and attachments are encrypted in transit over the internet. Both the sender and recipient email domains must have TLS enabled. This can be checked using https://www.checktls.com/
- **Secure corporately managed data repository and sharing platforms** (e.g. MS Teams; Google Docs)
- **Secure group email services** (e.g. CJSM: https://cjsm.justice.gov.uk/index.html)
- **Secure File Transfer Protocols**
- **Virtual Private Networks**

The above are examples, get advice from your organisation's information security or IT teams on secure methods of sharing available at your organisation and document these in the organisation's process documents.

Phone/virtual meetings/face-to-face meetings:

Information may be shared over the phone, in a virtual meeting, or at face to face meetings. Meeting attendance and distribution of content, e.g. meeting minutes or recordings, must be limited to those with a need to know. Sharing by telephone should be avoided unless the requirement is urgent and email is not practicable. Individuals should be aware of their surroundings and the presence of other individuals or voice recognition or 'Internet of Things' devices (e.g. virtual assistant apps like Alexa, Cortana, SIRI) to ensure they aren't overheard by those that should not have access to the information discussed.

Paper records:

Printed paper records must always be kept to a minimum and kept secure whether in the office, home or during transit. Organisations must adopt an appropriate policy surrounding the use and transfer of paper records. Appropriate security methods must be applied when storing or disposing of paper records.

4.3. Access controls and security

Both Parties will ensure that they have appropriate technical and organisational security measures in place to guard against unauthorised or unlawful processing of personal data and against accidental loss or destruction of, or damage to, personal data.

All personal data held by the Parties electronically will be stored in a secure network area with password protected entry and appropriate back-up functionality. The systems will be auditable so that it is possible for any auditor to establish who has accessed the system. All laptops, computers, and any other portable devices will be encrypted.

Any individual no longer required to have access will promptly have such access revoked by the line manager and Human Resources related to the relevant employer.

Both partners are NHS Digital's Data Security and Protection Toolkit (DSP) compliant. There is an expectation that partner organisations will either be working toward ISO 27001, the International Standard for Information Security Management, or a similar standard of security such as Cyber Essentials or Cyber Essentials Plus.

4.4. Outside UK processing

There is no intention to share data outside the EEA. Parties are responsible for ensuring that if information is processed or shared outside the EEA, that appropriate safeguards are in place and on condition that enforceable data subject rights and effective legal remedies for data subjects are available. These are for example, a legally binding and enforceable instrument between public authorities or bodies, binding corporate rules, and/or standard data protection contractual clauses.

4.5. Data quality

Each partner is responsible for ensuring the accuracy and relevance of the personal data that it processes and shares and must have clear processes in place for managing data quality.

Any party learning of the inaccuracy of personal data is responsible for informing the parties with whom that data has been shared.

4.6. Data breaches/incidents

All parties must have a clear policy and procedure regarding the reporting and handling of data protection breaches or data loss incidents. This must include assessing the level of risk to the data subject(s), as well as to make a decision on notifying the ICO within the statutory time frame of 72 hours.

If the incident may impact the processing of another party to this agreement, all relevant parties should be informed and appropriate coordination of the incident must take place. The decision to report the incident will lie with the data controller(s) of the information concerned. The parties agree to provide all reasonable and necessary assistance at their own expense to each other to facilitate the handling of any personal data breach in an expeditious and compliant manner.

It is confirmed that security breaches (including misuse or unauthorised disclosure) are covered by the partner's internal disciplinary procedures. If misuse is found there should be a mechanism to facilitate an investigation, including initiating criminal proceedings where necessary.

4.7. Retention and Disposal

Organisations are required by data protection legislation to document processing activities for personal data, such as what personal data is held, where it came from and with whom it has been shared. This Record of Processing Activity (ROPA) must include the retention period for the data.

Information must not be retained for longer than necessary for the purpose for which it was obtained. Disposal or deletion of personal data once it is no longer required, must be done securely with appropriate safeguards, in accordance with that organisation's disposal policies.

5. Signatures and Contacts

[redacted]

Director of Public Health London Borough of Islington

Date 28/02/2023

[redacted]

Director of Public Health London Borough of Camden

Date 28/02/2023

Contacts for Data Protection matters:

London Borough of Islington: Leila Ridley, Head of Information Governance and Data Protection Officer dp@islington.gov.uk or Joe Luxton, Data Protection Lead dp@islington.gov.uk

London Borough of Camden: Andrew Maughan, DPO, DPO@camden.gov.uk or Sarah Laws Data Protection Manager DPO@camden.gov.uk

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6. Appendices

6.1. Appendix 1

Shared Service will include the following:

1. **Smokefree Project Officer**

   The Smokefree Project Officer leads on the development of the smoke free agenda across Camden and Islington Councils. The main deliverables for the role include:

- Develop and coordinate implementation of the councils' tobacco control strategy in partnership with key stakeholders. Includes commissioning effective support for smokers to quit that also reaches those most at risk of poor health or health inequality; supporting cross-borough work on illicit tobacco and shisha; smokefree environments; and preventing young people from starting to smoke
- Manage the stop smoking service contract
- Plan and implement smokefree communications
- Review and update the smoking LCS specification
- Support the development and implementation of the NHS Plan for smoking cessation in secondary care
- Lead on/ contribute to the planning, design and analysis of quantitative and qualitative evaluation regarding local tobacco control issues
- Lead on conducting self-assessments of local activity across tobacco control and smoking
- Provide subject expertise to partners across the system
- Represent the councils in London-wide tobacco control networks.

2. **Commissioning Manager for Sexual and Reproductive Health**

The Commissioning Manager for Sexual and Reproductive Health is responsible for commissioning sexual health services across Camden and Islington. The main deliverables for the role include:

- managing the strategic commissioning of sexual health services
- implementation of the overall agenda around sexual health
- working, and co-producing the local approach, with service users and a wide variety of stakeholders, to commission high quality services that improve health and well-being outcomes for adults and families
- provide subject expertise to partners across the system

3. **Contract Support Officer (fixed term until April 2024, funded through the Rough Sleeping Grant in each respective Council)**

The Contract Support Officer was appointed to add capacity to the commissioning team and in response to the increased workload in relation to Rough Sleep Drug and Alcohol Treatment Grant funding. The main deliverables for the role include:

- Arranging and minuting all contract monitoring meetings including community drug and alcohol services, grant funded drug and alcohol services and the stop smoking service
- Maintaining an action log from contract review meetings for SM and Stop Smoking Services, including follow up with providers and commissioners
- Invoicing

- Managing performance monitoring of the community pharmacy naloxone pilot
- Supporting procurements

4. **Knowledge Team**

   The purpose of the shared Public Health Knowledge Team is to provide specialist analysis of resident feedback and insights to inform council strategies, programmes and practice. This will be done through:

- Strategic advice and inputs into council-wide data and insight processes, including Good Life Camden, Food Mission measurement framework, Islington Data and Insights Board and the next phase of Let's Talk Islington.

- Leading design, data collection and analysis for larger, strategic projects, such as the Covid resident engagement (surveys, focus groups, interviews) and inclusion health needs assessment.

- Providing quality assurance and training, mentoring and advising colleagues in programmatic roles, VCS organisations and community researchers on design, sampling, collection, analysis and interpretation, such as the forthcoming survey and focus groups in both boroughs on parental attitudes towards childhood vaccination.

5. **Data Manager**

The data manager will be responsible for handling, formatting and supplying the Camden intelligence team with datasets ready for analysis. This will include births and deaths data, hospital episode statistics, sexual health sensitive data. The data manager will also prepare any linked datasets required by the intelligence team e.g Adult social care and Healthcare data